

HealthConnect Week 8 project summary

Final analytics and decision support | Wilson Moses

What the source shows

The source contains 5,000 appointment records for 1,696 distinct anonymized patients. There are 4,737 completed appointments, including 2,314 attended visits and 2,423 no-shows. The no-show rate is 51.2% of completed appointments. Cancellations are separate: 263, or 5.3% of all records.

Measure	Result
No-show rate among completed	51.2%
Cancellation rate among all records	5.3%
Reminder coverage among all records	72.7%
No-show rate, 0-3 day lead	26.6%
No-show rate, 31+ day lead	63.9%
Missing distance values	90 (1.8%)

Main finding

No-show rate rises from 26.6% for bookings 0-3 days ahead to 63.9% for bookings 31 or more days ahead. The long-lead group accounts for 60.5% of no-shows. This is the clearest descriptive signal for a prospective confirmation-timing test.

Recommended action

Pilot supportive confirmation or reminder timing for appointments booked well in advance. Compare the approach with current practice. Track attendance, cancellations, successful contact, refilled slots and staff time. The reminder comparison is observational and does not prove effectiveness.

Week 7 transition

No Week 7 Data Analytics report or test result was present in the project materials reviewed. This package does not invent a Week 7 output, issue or refinement. Week 8 metrics were recalculated from the unchanged shared source and should be reconciled with the Week 7 handoff if it is available.

Cross-track contribution

The available Week 6 Data Science summary records a no-history logistic regression candidate with mean temporal validation AUC 0.6481. The analytics package provides KPI definitions and lead-time segments for interpretation and monitoring. Fresh validation and feature-timing checks remain necessary. The handoff is prepared for review; no human approval is claimed.

Readiness

Ready for final presentation as descriptive decision support. The file is synthetic, patients repeat across rows, associations do not establish causes, and the output is not deployed or suitable for clinical decisions.

Source: HealthConnect_Appointment_Data.csv and data dictionary. SHA-256:
1f2c40ae14833b46fe6e1a353446a0e038f19ae06434b37d98545b3e28697137